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Wells' Criteria for DVT

Calculates risk of DVT based on clinical criteria.

INSTRUCTIONS

- **Note**: The Wells' Score is less useful in hospitalized patients (Silveira PC, 2015).
- There are a few versions of this criteria with minor differences based on the study; this set is the most widely validated, based on Wells 2003.

When to Use | Pearls/Pitfalls | Why Use

0 points

Low risk group for DVT. “Unlikely” according to Wells’ DVT studies.

Copy Results | Next Steps

Calf swelling >3 cm compared to the other leg Measured 10 cm below tibial tuberosity	No 0	Yes +1
Collateral (nonvaricose) superficial veins present	No 0	Yes +1
Entire leg swollen	No 0	Yes +1
Localized tenderness along the deep venous system	No 0	Yes +1
Pitting edema, confined to symptomatic leg	No 0	Yes +1
Paralysis, paresis, or recent plaster immobilization of the lower extremity	No 0	Yes +1
Previously documented DVT	No 0	Yes +1

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ADVICE

- As with all clinical decision aids, the Wells' DVT criteria is meant to aid clinical decision making and not force management.
- The Wells' DVT criteria should only be applied after a detailed history and physical is performed.
- The Wells' DVT criteria should only be applied to those patients who have been deemed at risk for DVT. If there is no concern for DVT, then there is no need for risk stratification.

MANAGEMENT

Patients can be divided into "DVT unlikely" and "DVT likely" groups based on Wells score. An additional moderate risk group can be added based on the sensitivity of the d-dimer being used.

- A score of 0 or lower is associated with DVT unlikely with a prevalence of DVT of 5%.
  - These patients should proceed to d-dimer testing:
    - A negative high or moderate sensitivity d-dimer results in a probability <1 % and no further imaging is required.
    - A positive d-dimer should proceed to US testing.
      - A negative US is sufficient for DVT rule out.
      - A positive US is concerning for DVT; strongly consider treatment with anticoagulation.
- A score of 1-2 is considered moderate risk with a pretest probability of 17%.*
  - These patients should proceed to **high-sensitivity** d-dimer testing (moderate sensitivity d-dimer is not sufficient).
    - A negative **high-sensitivity** d-dimer is sufficient for rule out of DVT in a moderate risk patient with a probability of <1%.
    - A positive **high sensitivity** d-dimer should proceed to US testing.
      - A negative US is sufficient for ruling out DVT.

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- All DVT likely patients should receive US.
- D-dimer testing should be utilized to help risk-stratify these DVT-likely patients.
  - In DVT likely patients with negative d-dimer:
    - A negative US is sufficient for ruling out DVT, consider discharge.
    - A positive US should be concerning for DVT, strongly consider treatment with anticoagulation.
  - In DVT likely patients with a positive d-dimer:
    - A positive US should be concerning for DVT, strongly consider treatment with anticoagulation.
    - A negative US is still concerning for DVT. A repeat US should be performed within 1 week for re-evaluation.

***Group should only undergo d-dimer testing for rule out without ultrasonography if a high-sensitivity d-dimer is being used.**

CRITICAL ACTIONS

- No decision rule should trump clinical gestalt. High suspicion for DVT should warrant imaging regardless of Wells score.
- The Wells' Criteria for DVT is utilized for the workup of DVT. The presence of DVT is critical to the evaluation of possible PE, and if PE is on the differential, alternative decision aids such as the Wells' PE or PERC Rule should be entertained.

Content Contributors

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Related Calcs

- Geneva Score (Revised)
  Objectifies risk of PE, like Wells' score.
- Villalta Score
  Stratifies severity of post-thrombotic syndrome in lower extremity DVT.
- Wells' Score for PE
  Objectifies risk of pulmonary embolism.

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